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The Minister of VWS
P.O. Box 20350
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Date

November 28, 2023

Our reference

2023-226557

Your letter dated

July 4, 2023

Your reference

3627862-1050433-DICIO

Subject

Draft bill on the accessibility of data in urgent care

Dear Mr. Kuipers,

In your letter dated July 4, 2023, the Authority for Personal Data (AP) was consulted pursuant to Article 36, fourth paragraph, of the General Data Protection Regulation (GDPR) regarding the draft bill containing rules for making data directly accessible for the provision of urgent care (hereinafter: the draft).

The AP has a number of comments on the draft and advises that these should be taken into account.

Scope of the draft

This draft bill regulates that care providers designated by general administrative order must ensure that healthcare professionals under their authority make data designated by general administrative order directly accessible, unless good care practices oppose this or the personal privacy of another is compromised. It is up to the care providers themselves to determine how to comply with this obligation: this can, but does not have to, be done by making the data available through an electronic exchange system (EUS) as referred to in the Act on Additional Provisions for the Processing of Personal Data in Healthcare (Wabvpz).

1 Article 4, first paragraph.

2 Article 5.

3 Essentially the National Switch Point (LSP).

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The data that have been made (mandatory) directly accessible may, under this draft bill, only be requested by healthcare professionals who fall under the categories of care providers designated by general administrative order in the case of urgent care. At that time, the client has the option to limit the request for data unless there is a "code black."

Advice

Basis: consent or legal obligation?

In the explanatory memorandum, it is established that the necessary data about a client in the case of

urgent care must be directly accessible to provide this care adequately. It then elaborates on the question of which basis has been designated for data exchange for the purpose of urgent care: consent or legal obligation?

It is noted that the (existing) requirement that a patient must give prior consent for the provision of data by their healthcare provider via an EUS does not necessarily hinder the availability of this data in urgent care. However, according to the explanatory memorandum, there are circumstances that prevent the (implicit) ambition to obtain the required consent from "almost all people in the Netherlands":

1. A large portion of the Dutch population has not yet indicated whether they agree to the availability of their data via EUS.

2. Consents that have been given are not sufficient for all urgent care processes, for example, because the consent does not provide for exchange with a hospital.

3. Communication campaigns urging patients to give consent require significant efforts from care providers, healthcare professionals, and VZVZ and have proven to be insufficiently effective so far.

4 Articles 7, 8, and 9, first paragraph.

5 Article 9, first paragraph, introductory text, and under b. "Limiting" must also be understood to mean: completely blocking.

6 Article 10. "Code black" refers to a nationally declared crisis situation in which the capacity in intensive care is so insufficient in relation to the required number of facilities that triage based on medical substantive considerations, considerations in the medical domain, or the professional judgment of the responsible healthcare provider no longer offers a solution.

7 For an extensive analysis of the relevant legal framework, see the AP advice of February 25, 2021 (case number z2020-17310): Advice on electronic consultation of COVID-19 data | Authority for Personal Data.

8 Explanatory memorandum p. 7-9.

9 Explanatory memorandum p. 8.

10 See Article 4, eleventh paragraph, GDPR.

11 For a long time, only consent was requested from clients for the availability of:

- the Professional summary by general practitioners for the purpose of coverage by another doctor - for example, by a substitute general practitioner at the emergency department. The Association of Care Providers for Care Communication (hereinafter: VZVZ) indicates that in May 2023 approximately 9.7 million people granted consent for this.

- Recent medication data by pharmacists to (substitute) general practitioners (for example, at the emergency department), (outpatient) pharmacies, and doctors in hospitals and other institutions. VZVZ indicates that in May 2023 approximately 14.1 million people granted consent for this.

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4. The online provision of (specified) consent by the patient has so far proven to be a (too) high threshold.

What is presented in the explanatory memorandum regarding this must be considered plausible.

Furthermore, given the nature of "emergency care" and the necessity to have relevant data available, it is unlikely that a patient genuinely wishes for their data not to be accessed by emergency care. In such a case, an opt-out system as proposed indeed seems more appropriate.

Consideration of existing objections registered with LSP

The draft does not take into account the situation where a patient has previously explicitly refused consent for data exchange via the National Switch Point (LSP) and has registered this objection.

In that case, this refusal must also be respected in emergency situations.

The AP advises adding this exception to Article 5, paragraph 1.

"Considerable objections"

Article 9, paragraph 1, introductory phrase and under b, stipulates - insofar as relevant here - that a healthcare provider may not request data if the client has considerable objections against this.

With the mere addition of the adjective "considerable," it is unclear what this should be weighed against. Apart from this, it remains completely unclear in the explanatory memorandum why this addition would be necessary at all. For a valid opt-out, expressing an objection (or refusal) is sufficient.

The AP advises removing "considerable" from Article 9, under b.

Treatment prohibition must also be included in the dataset

Article 2 of the draft stipulates that - insofar as relevant here - data necessary for providing good emergency care will be designated by general administrative order (amvb).

For some patients, a so-called treatment prohibition has been recorded in the medical file. Such a treatment prohibition is particularly relevant in the context of emergency care: often, the patient will not be able to express their wishes on the spot. In the case of an applicable treatment prohibition, consulting (other) medical data is, of course, no longer necessary.

12 Article 1, paragraph 1.

13 But not impossible.

14 The draft only provides an option to object to the request for data, not against making the data available. See also the following paragraph.

15 See for example Written Treatment Prohibition (nvve.nl).

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The AP advises that the treatment prohibition - if present - should also be made requestable in accordance with Article 4, paragraph 1, so that this can be taken into account when providing emergency assistance and to prevent, if possible, that (other) medical data is retrieved despite an applicable treatment prohibition.

Security/architecture

Article 4, paragraph 1, obliges the healthcare provider to make the data designated by general administrative order (amvb) "immediately requestable" for emergency care within 24 hours after they have received it.

This information must therefore be continuously available for all patients (as anyone can end up in an emergency situation), while it is only used in the exceptional situation of urgency. This potentially creates a significant security risk. The explanatory memorandum does not address this.

The AP advises that the explanatory memorandum should pay attention to the security risks of this continuous availability of the intended data.

Definition of "emergency care"

In the draft, the term "emergency care" has been chosen instead of the already existing term "acute care." The reason for this, according to the explanatory memorandum, is that the proposed definition aligns with the medical content judgment of the healthcare provider, while the definition of "acute care" in the Healthcare Quality, Complaints, and Disputes Act (Wkkgz) instead aligns with the experience of a client at the time of the care request.

However, this is incorrect. "Acute care" is defined in the Wkkgz as "care related to an experienced or

observed possible serious or imminently life-threatening situation [...] [emphasis AP]." The objection that the Wkkgz is based on the client's experience rather than the medical content judgment of the healthcare provider does not apply.

But evidently, the intention is that it can only concern the medical content judgment of the healthcare provider, and thus not also the client's experience. The question then is whether the definition of "acute care" in the Wkkgz is actually correct.

16 What is meant by 'immediate' depends on the specific case and is left to the judgment of the involved healthcare providers [...] It is desirable that the [...] data is provided without delay, but in any case within a timeframe of a maximum of half an hour" (Explanatory Memorandum p. 12).

17 Article 1, paragraph 1, Healthcare Quality, Complaints, and Disputes Act.

18 Explanatory Memorandum p. 22.

19 Article 1, paragraph 1, Healthcare Quality, Complaints, and Disputes Act.

20 It appears that in this definition the subjective element from the definition of "acute care request" from the RVZ advice 2003 has been adopted while the included definition of "acute care" does not contain this element (Advice Acute Care, Council for Public Health and Care, 2003, p. 5).

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In addition, it should be noted that the introduction of the concept of "urgent care" alongside the previously legally defined "acute care" does not enhance the readability and accessibility of the law. Both terms, in common usage, have the same meaning.²¹ Generally, it is better to strive for uniformity and to use one term.²²

The AP advises that appropriate attention be given to the above.

Workload AP

It is likely that the concept will require the AP to make limited additional efforts.²³ This effort has not yet been accounted for in the AP's funding. This will need to be addressed in consultation with the Minister for Legal Protection - who is responsible for the budget from which the AP is financed - prior to the entry into force. For this reason, a copy of this advice is sent to the Minister for Legal Protection.

Disclosure

This advice will be published on the AP's website within two weeks.

Yours sincerely,
Authority Personal Data,

Aleid Wolfsen
Chairman

21 Both "acute" and "urgent" mean "requiring an immediate solution."

22 See also Guideline for Legislation 3.7.1.

23 See also Explanatory Memorandum p. 44-45: "[H]ere it applies that these tasks are in line with the tasks that the AP already has in the context of the GDPR and UAVG. This means that this task does not need to have much (personnel) impact for the AP."